



POLICY

Clinical Assessment

Scope (Staff):	All Clinical Staff
Scope (Area):	CAMHS

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

This document should be read in conjunction with this [disclaimer](#)

Only use this box if relevant (e.g. clinical document or available on an external website)

Contents

Aim	2
Risk.....	2
Background.....	2
Definitions	2
Principles / Key points.....	3
Prior to Assessment.....	4
Assessment – General.....	5
Collaborative development of a TSDP	8
Reviews – General.....	9
Reviews – Specific Circumstances	9
Documentation – General	11
Documentation – Specific Circumstances	12
Communication of Assessment results	12
Children and families who decline assessment.....	12
Evaluation	13
Auditing compliance with the policy.....	13
Training.....	13
Appendix 1: CAMHS Clinical Assessment in Focus – Aboriginal Children.....	17

Aim

To support staff to provide comprehensive, individualised assessments that ensure holistic care planning.

Risk

If clinical assessment is not undertaken there is a risk that holistic care planning for children is not provided.

Background

This policy covers assessment and diagnosis, formulation, and development of management plans.

This Policy must be read in conjunction with the [Mental Health Act \(WA\) 2014](#) ('MHA')², [Chief Psychiatrist WA Standards of Clinical Care 2015](#)¹, CAMHS [Consent guideline](#), and the CAMHS [Risk Assessment and Management policy](#).

Definitions

Assessment: 'Process by which the characteristics and needs of a child and their family or carer are evaluated and determined so that they can be addressed. The assessment forms the basis of treatment, care and recovery planning'¹.

Child: an infant, child, adolescent or young person under the age of 18 years.

Capacity: A person has the capacity to make a decision about a matter relating to themselves if the person is able to:

- understand any information or advice about the decision;
- understand the matters involved in the decision;
- understand the effect of the decision,
- weigh up the above factors for the purpose of making the treatment decision; and
- communicate the decision in some way' (MHA 2014, s. 15)².

Family: any parent, legal guardian or identified carer of the child. For a child of Aboriginal or Torres Strait Islander descent, this may be a person from their kinship group who is recognised under customary laws and traditions as their carer. See the definition of 'close family member' (MHA 2014, s. 281)².

Consent: an agreement from the child / young person and/or their family / carer based on an understanding of the implications of a particular activity or decision and the likely consequences.

Formulation: a case formulation can be defined as a set of 'hypotheses about the causes, precipitants, and maintaining influences of a person's psychological, interpersonal, and behavioural problems'¹.

Key stakeholder: professionals involved in the care of the child or support of the family, including but not limited to referrer/referring agency, school, group home, Department of Communities, Child Protection and Family Support, General Practitioner (GP).

Treatment Support and Discharge Plan (TSDP) (nee Management Plan): a recovery-informed plan, which includes clear identification of issues, shared goals, actions and timeframes, developed in collaboration with the child and family. [SSCD Forms - Treatment, Support and Discharge Plan](#).

Multidisciplinary Team (MDT) Clinical Review: a regular formal appraisal of care that evaluates planned or provided treatment and directs changes in treatment to best accommodate child and family needs.

Positive Risk Management: person-centred approach that involves recognising and respecting an individual's right to make informed choices about their own lives, their preferences, aspirations, values, and needs including the decision to take risks.

Principles / Key points

- The values of recovery-oriented, person-centred, and trauma-informed care, that is culturally competent and developmentally appropriate underpins care provided in CAMHS³.
- Triage assessment is completed on webPSOLIS at initial referral / presentation to CAMHS. If risk is identified a CRAMP is completed with the consumer, family if possible. See [webPSOLIS Triage Referral work instruction](#).
- Assessment is a continuous process which can never be exhaustive. 'A comprehensive assessment may occur over a number of treatment sessions'¹.
- Assessments should aim to:
 - support engagement with the child / young person and family/ carer;
 - gather comprehensive information to allow the clinician to analyse facts;
 - communicate recommendations to support collaboration between the clinician and the child and family; and
 - be transparent and empowering.
- Where possible and safe, assessments should be conducted at a time and place convenient to the child and family.
- Assessments must be informed by the psychosocial, developmental, and cultural context of the child within the framework of recovery-oriented practice:
 - Assessments are a collaborative engagement between the expertise of the clinicians and the lived experiences and personal expertise of the child and family.
 - The child and their family must be actively and sensitively involved in the assessment process. This should lead to the co-production of the individual, recovery focussed TSDP.
- The child / young person has the right to request assessment from a clinician of their preference with an understanding of their needs.

- Assessments (clinicians) must take into account cultural and social diversity, including considerations around Aboriginality, gender, ethnicity, pregnancy or maternity, disability, neuro divergence, religion, spirituality and/or belief, sexual diversity and gender identity, socioeconomic status, trauma history, medical condition and age.
 - For further information around assessment and Aboriginal children see Appendix 1: CAMHS Clinical Assessment in focus – Aboriginal children
- Appropriate assistance and reasonable adjustments must be provided for children and their families who are not proficient in speaking, reading or writing English, or who have a communication disability, hearing impairment, or learning disorder as per the [WA Health Language Services Policy](#).
- Where feasible, all assessment instruments or inventories used with Aboriginal or culturally and linguistically diverse children should be culturally appropriate and culturally validated. The quality of the clinical care will be enhanced through the use of consultation and communication with key stakeholders about the outcomes of the clinical assessment.
- For children who are assessed as being at risk of harm to themselves or others or who may be at risk of becoming violent or aggressive:
 - Safety plans must be developed in collaboration with the child, family and the multidisciplinary team².
 - Positive risk management recognises that all decisions carry some element of risk. This must be integral to the process of safety planning. This approach builds on the consumer's strengths and enhances their recovery, is based on a trusting therapeutic relationship, and uses least restrictive practice¹.
 - See CAMHS [Risk Assessment and Management policy](#).

Prior to Assessment

- The child and family should be provided with information about the service prior to the assessment, wherever possible. This aims to support informed decision making and engagement with the service.
- Where a child seeks a mental health assessment without the support of their family, a decision about whether to progress with the assessment without parental authority must be made. This decision must be based on the child's clinical need, family circumstances, and capacity as determined by the clinician in consultation with the multidisciplinary team. See [Assessing Capacity to Consent in Minors, a clinical handbook](#).
- Prior to conducting an assessment, clinicians must review the young person's mental health history (including any information available on PSOLIS). This may include consultation with the services or professionals who have been involved in the care of the child and support of the family, especially where the child has been recently seen by the services / professionals.
- At the outset of the interview, before the assessment commences, the clinician must:
 - determine if the child is aware of the fact of and reason for referral,

- develop a shared understanding of the nature and purpose of the interview,
- establish consent in line with CAMHS [Consent guideline](#)
 - consider capacity to consent if relevant,
 - for further guidance see [Assessing Capacity to Consent in Minors, a clinical handbook](#).
- inform the child and family of the records that are kept and explain the limits and constraints of confidentiality (see CAHS [Confidentiality, Disclosure and Transmission of Health Information policy](#)).
- provide the child / young person and family / carer with an explanation of their rights and responsibilities (MHA 2014, s. 243-246) and the [MHA 2014 Charter of Mental Health Care Principles](#) – see CAMHS [Patient Rights Under the Mental Health Act policy](#) and CAMHS [Right to Communication and Receiving Visitors policy](#)
- ensure the child and family has received the welcome pack including:
 - service information,
 - advocacy information,
 - how to make compliments and complaints and provide feedback.
- **Community CAMHS:** The process for triage and assessments is described in the CAMHS [Community CAMHS Entry protocol](#).

Assessment – General

At the beginning of any episode of care within CAMHS, a comprehensive biopsychosocial clinical assessment must be completed. The Statewide Standardised Clinical Document (SSCD), [SSCD CAMHS Initial Assessment form](#) must be used to guide, develop, and document the following:

- Diagnosis, according to ICD10 AM, or Diagnostic Classification of mental health and developmental disorders in Infants and young children DC: 0-3;
- Formulation, identifying vulnerabilities, strengths, and protective factors;
- Collaborative development of a goal-based TSDP (plans for discharge are included as part of the initial assessment).

The assessment must include:

- Collation of 'medication information' including:
 - current and past medications;
 - over the counter medications;
 - complementary/alternative/herbal medications;
 - allergies;
 - adverse reactions;
 - misuse;
 - side effects; and
 - medication adherence¹³.
- Completion of Mental Health National Outcomes and Casemix Collection (NOCC) assessments Health of the Nation Outcome Scale Child and Adolescent (HONOSCA) and the Child Global Assessment Scale (CGAS);

- Completion of Strength and Difficulties Questionnaires (SDQs) by the family (for Community CAMHS, this occurs at the admission / activation stage);
- Consideration of information from the child, family, or liaison with other key stakeholders and from the child's medical record (MHA 2014, s. 49, 80). Any previous mental health assessments that have been documented within the medical record must also be considered;
- Completion of a risk assessment, commonly referred to as RAMP, in accordance with the CAMHS [Risk Assessment and Management policy](#). In Community CAMHS, risk assessment is undertaken at the initial contact appointment but may be reviewed as part of further assessments if clinically indicated;
- An assessment of whether the child needs to be assessed under the MHA as an inpatient or in the community;
- An interview with the child, an interview with the parents, and a family interview, as appropriate; and
- Use of tests and instruments where clinically indicated.

Assessment - Specific Circumstances

For Aboriginal children, see [Appendix 1 'CAMHS Clinical Assessment in focus - Aboriginal children'](#).

CAMHS Ward 5A

If another CAMHS service has completed an initial assessment on the SSCD form within 24 hours (unplanned admissions) or one working day (planned admissions) of referring the young person to Ward 5A, the initial assessment should not be repeated by Ward 5A. Staff must document this in the young person's progress notes as per the CAMHS Ward 5A and Nickoll Ward [Admission to CAMHS Inpatient Service procedure](#).

In all other cases, an initial assessment, risk assessment, Comprehensive Admission and Screening Tool and initial plan (including goals of admission) must be completed by a Registrar within 4 hours of admission to Ward 5A (see the [Admission to CAMHS Inpatient Service procedure](#)). A verbal handover regarding risk and initial plan must be communicated to the Shift Coordinator in an ISOBAR format (see CAMHS [Clinical Handover guideline](#)).

A comprehensive and documented physical health assessment must be completed by a Medical Practitioner within 12 hours of admission.

In all new episodes of care, children are to have a face to face review by a Consultant Psychiatrist within 24 hours of admission (See CAMHS [Physical Health Assessment and Monitoring policy](#)).

Children who are prescribed psychotropic medication

A comprehensive and documented physical health assessment must occur for all children who are prescribed psychotropic medication using the standardised CAMHS Physical Examination Form.

Further information can be found at:

- CAMHS [Physical Health Assessment and Monitoring policy](#) and the
- CAMHS [Medication Safety Toolbox: Psychotropic Medication Physical Health Monitoring Handbook](#)
- CAMHS [Off-Label Psychotropic Prescribing Guide for CAMHS Clinicians guideline](#)
- [Consent for Medication Use \(MR505.00 / MR859.01\)](#).

Children who may be at risk

Clinicians should adopt a structured clinical judgement approach when it comes to assessment of children who are at risk. The SSCD Risk Assessment and TSDP form is one component of the assessment process. *'It involves the clinician making a clinical judgement about risk, based on a combination of the evidence base for risk factors, individual patient assessment including the child and family's view of their own experiences and circumstances, together with clinical experience and knowledge of the child and family'*⁴. Clinicians must maintain awareness that risk is dynamic and can change over very short timeframes.

In collaboration with the child / young person (where possible) and the family / carer a risk assessment must be conducted and is to encompass:

- a detailed evaluation of all aspects of risk, including sexual safety, risk of harm to self, risk of harm to others, vulnerability, and suicidal behaviour and ideation;
- a comprehensive diagnostic assessment and formulation:
 - in cases of high risk, there should be consultation with a senior clinician and/or psychiatrist,
 - where children are suicidal a psychiatric diagnostic assessment and formulation must be completed; and

a thorough determination of the psychosocial circumstances and context contributing to the clinical presentation. This involves an assessment of the family's ability to safeguard their child and contain risk, and consideration of:

- historic, cultural and current social and psychiatric factors;
- child's current mental state;
- history of aggression or violence;
- history of abuse
- history of self-harm;
- developmental history;

history concerning relevant medical or organic issues (for example trauma or drug interaction);

triggers;

usual context of violence or aggression (including the young person's input about their feelings and behaviours when acting aggressively or violently); and

previous response to treatment.

For more information regarding assessment of children who may be at risk see:

- CAMHS [Risk Assessment and Management policy](#)
- [Chief Psychiatrist's Standards for Clinical Care](#)
- [DoH Principles and Best Practice for the Care of People Who May Be Suicidal](#)⁴.
- DoH [Principles and Best Practice for the Care of People Who May Be at Risk of Exhibiting Violent or Aggressive Behaviour](#)⁵.

Requests for community mental health status assessments⁶

Requests for community mental health status assessments must be managed in accordance with the:

- DoH MP 0099/18 [Community Mental Health Status Assessments: Role of Mental Health Clinician Policy](#);
- CAMHS [Home and Community Visits procedure](#).

If the request is urgent, the person requiring assessment should be put in contact with CAMHS Crisis Connect (1800 048 636).

Collaborative development of a TSDP

The initial assessment (in conjunction with the CRAMP / RAMP and NOCC results) must guide the development of the functional initial management plan which will be reviewed as further information is obtained. Clinicians must discuss the findings of the assessment with the child / young person and family / carer and collaboratively develop a TSDP. Where multiple clinicians are involved in the child's care, all efforts must be made to involve them in the development of the TSDP and the [Shared Care in CAMHS guideline](#).

The TSDP must:

- be developed using the SSCD Initial Assessment form and PSOLIS / webPSOLIS TSDP;
- be holistic and recovery informed;
- include goals and strategies reflective of outcome measure scores; and
- be signed by the child and family and a copy given to them.

For CAMHS Ward 5A, the TSDP must:

- be commenced within 24 hours of admission and reviewed at the first MDT meeting;
- Include achievable goals that are clearly related to the discharge plan; and
- be informed by the comprehensive admission and screening tool and the young person's individual crisis support plan.

For children in Ward 5A Pod 1, an exit strategy must also be included in the TSDP. A review of the appropriateness of risk containment must be carried out for each child.

Reviews – General

The results of the comprehensive assessment, including formulation, diagnosis, recommendations, results of outcome measures, and goal based TSDP /management plan (including plans for discharge), must be reviewed by the multidisciplinary team (MDT). Staff should refer to the CAMHS [Multidisciplinary Team Clinical Reviews policy](#) for guidance regarding the timing and structure of MDT reviews.

Reviews – Specific Circumstances

CAMHS Ward 5A

The following must occur in addition to the general review section above.

Review and further development of TSDP completed within the first 48-72 hours of admission:

- The child and family must be consulted and given the opportunity to be involved in the review and decision-making process.
- Inclusion of clear therapeutic goals linked to the discharge plan.
- The child's treatment progress, progressive risk, continuing need for admission and location within the unit must be reviewed on a routine, ongoing basis. Review may be included as part of:
 - Clinical reviews,
 - Daily clinical handover meetings, and
 - MDT review meetings
- For children in Pod 1, reviews must be conducted every 24 hours, with clear documentation of indicators for transfer to less intensive environment.
- All reviews of admission must be documented in the medical record, and the child's management plan must be updated as required.

- The child's TSDP must be updated following each review, printed and provided to the child and/or family, signed by them, and filed in the medical record.
- Timeframes/processes for review of involuntary admission:
 - The [Mental Health Tribunal](#) must review an Inpatient Treatment Order (ITO) relating to a child within 10 days of the child's admission (MHA 2014 s. 386).
 - A child can be detained at the hospital under an ITO for a maximum period of 14 days (MHA 2014 s. 87). At the end of this period, the Order may be continued following review.
 - As a minimum the child who is admitted under an ITO must be examined by a psychiatrist for the purpose of reviewing their involuntary status within 7 days before expiry of the current Order (s. 89), and at least every 14 days in total.
 - If the psychiatrist determines that the child still requires treatment on an involuntary basis, they must continue the ITO using [Form 6C – Continuation of an inpatient treatment order](#) (MHA 2014 s. 89)
 - If the psychiatrist does not continue the order, the ITO expires at the end of the current 14-day period.
- Progressive risk must be assessed as per the CAMHS Inpatient Unit [Progressive Risk Monitoring work instruction](#).
- An initial MDT review must be conducted as soon as possible and within 48-72 hours of admission
- Further MDT reviews should occur at least weekly, as well as ad hoc reviews prompted by significant changes in clinical presentation and risk.
- Discharge MDT reviews should, where possible, include the follow up service at the point of discharge from the inpatient unit.
- Clinical reviews should be recorded in PSOLIS / webPSOLIS.
- Refer to the CAMHS Inpatient Unit [Admission to Pod 3 - Traffic Light Assessment work instruction](#) for criteria regarding whether or not the child should be cared for in the low acuity part of the Inpatient Unit.

See CPG 9.13 regarding reviews of long-term voluntary patients

- Reviews of long-term voluntary admissions (MHA 2014 s. 404-408)
- Applications for reviews by the Mental Health Tribunal can be made after 3 months of inpatient stay

For children who may be at risk

Safety Plans must be developed collaboratively between clinicians, the consumer (where possible), their family and their personal support person. The Safety Plan can

be incorporated as part of the treatment, support and discharge plan or as part of a crisis plan in PSOLIS / webPSOLIS.

The Safety Plan must be revised and updated at points of significant transitions in care, change/deterioration in clinical state, post-incident, when safety arrangements may need to be reviewed, and at other times that are recognised to be periods of heightened risk. For inpatients or previous inpatients these times include; first week of admission, during the immediate pre-discharge period, after going missing from hospital, the first week after the discharge period, following self-discharge and following discharge from non-local inpatient units.

Documentation – General

All clinical activity related to contact with the child, family and stakeholders must be documented in the medical record.

Key documentation requirements relating to clinical assessment include (but may not be limited to):

Clinical activity	Action
Consent and determination of capacity	Document in the medical record / or scan to digital medical record (DMR)
SSCD CAMHS Initial Assessment Form	Document in the medical record / DMR
Safety planning / Risk assessment and management	Document in the medical record as per the Risk Assessment and Management policy , webPSOLIS, CRAMP (can incorporate the safety Plan), SSCD, TSDP Forms
Results of psychometric testing and other diagnostic instruments	Document filing of results in the raw data file or medical record) / DMR (See CAMHS Medical Records Management procedure).
NOCC assessments	Enter into the Psychiatric Services Online Information System (PSOLIS). Print summary of results from PSOLIS and file in the medical record.
SDQ forms	Enter into PSOLIS. Print from PSOLIS and file in the medical record.

Information about a child received via phone call from a carer, family member or other person	Record the information in the medical record Add an event on PSOLIS / webPSOLIS
Treatment Support Discharge Plan (TSDP)	Complete in PSOLIS clinical alert if necessary and webPSOLIS, Signed copy to be printed and filed in the medical record.

Documentation – Specific Circumstances

In cases where the child and family, or significant others (e.g., Aboriginal community elders), are not involved in the collaborative development of a TSDP, reasons for this must be clearly documented in the child's medical record. Attempts to continue to engage collaboratively must be both ongoing and recorded.

For children who may be at risk or at risk of becoming violent or aggressive, safety plans must be completed and documented in the medical record/clinical notes. See the CAMHS [Risk Assessment and Management policy](#).

Communication of Assessment results

Clinicians must communicate results and recommendations of the assessment and any specific tests to the child and family as per the [Shared Care in CAMHS guideline](#).

Where relevant, and with the consent of the child and family, the clinicians must communicate the findings and recommendations of the assessment to key stakeholders. The timing and urgency of communication should take into account risk issues (see CAHS [Confidentiality, Disclosure and Transmission of Health Information policy](#)). Content or type of information to be released to agreed key stakeholders should be discussed with the child and family where appropriate. Issues of confidentiality must be considered. This discussion must be documented in the medical record. Information should only be communicated that is relevant to the particular service and/or to the clinician that the information is being given to. Discretion around particularly sensitive information should be used. Consent for the Release and Exchange of Information forms must be completed and signed, see CAMHS [Consent guideline](#) and the CAMHS [Shared Care in CAMHS guideline](#). Copies of all letters sent to key stakeholders must be kept in the medical record.

Children and families who decline assessment

The reasons for the child or family declining the assessment must be explored and attempts made to encourage the engagement of the child and family in assessment and treatment. The risk of declining an assessment should be considered and if necessary, contact should be made with the family and referrer, see CAMHS [Clinical Engagement policy](#).

Alternative clinical mental health services must be discussed with the child, family and referrer. Children and families who decline assessments must be reviewed at an MDT meeting or SBAR. Reasons given for declining assessment, attempts at engagement and alternative treatment options offered must be documented in the medical record. See CAMHS [Multidisciplinary Team Clinical Reviews policy](#).

Evaluation

Child and parent feedback on service

Each child and family must be encouraged and assisted to give feedback regarding the assessment process through both formal and informal avenues. Information from the CAMHS [Your Experience of Service \(YES\) and Carer Experience \(CES\) Surveys](#) is used to gather formal feedback from young people and families. Results and follow-up actions are tabled and discussed regularly at team, directorate and CAMHS wide governance meetings and communicated to children and families.

Auditing compliance with the policy

Compliance with this policy must be audited in alignment with the CAMHS Safety and Quality Audit Schedule.

Training

Assessments are conducted by appropriately qualified, culturally competent staff, supported by training opportunities, according to clinical best practice.

Clinical staff must complete the following training:

- Clinical skills training and Working Effectively with Cultural and Linguistic Diversity as part of CAMHS orientation;
- CAMHS Classification Skills training (CST), (Module 6 NOCC training on MyLearning and/ or PSOLIS training);
- Cultural Competency Training, Diversity WA (on-line training).

References and related external legislation, policies, and guidelines
1. Chief Psychiatrist's Standards for Clinical Care , November 2015 and Clinicians' Practice Guide to the Mental Health Act 2014 . Ed3.3; June 2024.
2. Mental Health Act (WA) 2014 ('MHA')
3. Australian Health Ministers' Advisory Council. A National Framework for Recovery-oriented Mental Health Services. Guide for Practitioners and Providers. Commonwealth of Australia; 2013.

4. DoH MP 0051/17: Language Services policy
5. MHA 2014 Charter of Mental Health Care Principles
6. Department of Western Australia Health. Principles and Best Practice for the Care of People Who May Be Suicidal ; 2017.
7. DoH 2024: Principles and Best Practice for the Care of People Who May Be at Risk of Exhibiting Violent or Aggressive Behaviour .
8. DoH MP0099/18: Community Mental Health Status Assessments: Role of Mental Health Clinician Policy
The Vancouver style referencing is as per CAHS Library and Information Service.

Related CAHS internal policies, procedures and guidelines
CAMHS Consent guideline
CAMHS Risk Assessment and Management policy
webPSOLIS Triage Referral work instruction . (permissions required for the webPSOLIS page)
CAHS Confidentiality, Disclosure and Transmission of Health Information policy
CAMHS Patient Rights Under the Mental Health Act policy
CAMHS Right to Communication and Receiving Visitors policy
CAMHS Community CAMHS Entry protocol
Ward 5A and Nickoll Ward Admission to CAMHS Inpatient Service procedure
CAMHS Clinical Handover guideline
CAMHS Physical Health Assessment and Monitoring policy
CAMHS Medication Safety Toolbox: Psychotropic Medication Physical Health Monitoring Handbook
CAMHS Off-Label Psychotropic Prescribing Guide for CAMHS Clinicians guideline
CAMHS Home and Community Visits procedure
CAMHS Shared Care in CAMHS guideline
CAMHS Multidisciplinary Team Clinical Reviews policy

CAMHS Inpatient Unit Progressive Risk Monitoring work instruction
CAMHS Inpatient Unit Admission to Pod 3 - Traffic Light Assessment work instruction
CAMHS Medical Records Management procedure
CAMHS Clinical Engagement policy
CAMHS Your Experience of Service (YES) and Carer Experience (CES) Surveys policy

Useful resources (including related forms)
SSCD, TSDP and MHA Forms (printable and writable forms)
Assessing Capacity to Consent in Minors: A Clinical Handbook . Child and Adolescent Health Service, Child and Adolescent Mental Health Services; Pace, G and Caunt J Nadine, 2015.
SSCD CAMHS Initial Assessment form
Consent for Medication Use (MR505.00 / MR859.01)
Form 6C – Continuation of an inpatient treatment order
Adams Y, Drew N, Walker R. Principles of Practice in Mental Health Assessment with Aboriginal Australians. In: Dudgeon P, Milroy H, Walker R, editors. Working Together: Aboriginal and Torres Strait Islander Mental Health and Wellbeing Principles and Practice [Internet]. 2014 2nd ed. Perth: Dept Prime Minister and Cabinet, Telethon Institute for Child Health Research, Kulunga Research Network and the University of WA.
Blue's Clues Making a diagnosis and using ICD 10 for coding purposes in CAMHS
CAHS Aboriginal and Torres Strait Islander Patient Care and Cultural Learning Guidelines
CAMHS Education and Training Guides (Clinical Handbook): General Issues in Clinical Work in CAMHS Interviewing Children and Families Risk Assessment in CAMHS The Present State Examination of the Child and Adolescent Selected Developmental Theories and Introduction to Ways of thinking about Child and Adolescent Mental Health

Introduction to Formulation and Treatment Planning
Consumer Handbook to the Mental Health Act 2014
Australian Mental Health Outcomes and Classification Network (AMHOCN).
CAHS Child Safeguarding and Protection Policy
ACRT (Metro) WI
CAMHS Teams Entry Protocols: • Community CAMHS • CAMHS Inpatient Unit (IPU) Entry Protocol • Complex Attention and Hyperactivity Disorders Service • Eating Disorders Service • Pathways • CAMHS Crisis Connect Entry Protocol

This document can be made available in alternative formats on request.

File Path:	CD/25/79304		
Document Owner:	ICAMHS Director Clinical Services		
Reviewer / Team:	Heads of Department, Professional Leads, CAMHS Policy Team, SSAMHS, Acute Care Response Team (Metro), Learning and Development		
Date First Issued:	August 2014	Last Reviewed:	19/02/2025
Amendment Dates:	July 2020	Next Review Date:	20/02/2028
Approved by:	ICAMHS Director Clinical Services	Date:	December 2024
Endorsed by:	Safety & Quality, Audit, Risk & Compliance Executive Committee	Date:	05/02/2025
Aboriginal Impact Statement and Declaration (ISD) 4187		Date ISD approved:	21/03/2025
Health Equity Impact Statement and Declaration (ISD)		Date ISD approved:	21/03/2025
Standards Applicable:	NSQHS Standards:  Child Safe Standards: 2, 3, 4, 5, 7, 10		

Printed or personally saved electronic copies of this document are considered uncontrolled



Healthy kids, healthy communities

Compassion

Excellence

Collaboration

Accountability

Equity

Respect

Neonatology | Community Health | Mental Health | Perth Children's Hospital

Appendix 1: CAMHS Clinical Assessment in Focus – Aboriginal Children

Mental health from an Aboriginal and Torres Strait Islander perspective encapsulates multidimensional factors including emotional, social and spiritual well-being, connection to country, culture, ancestry, family and community⁴. CAMHS recognises that Aboriginal children are particularly vulnerable to experiencing mental illness as a consequence of inter-generational trauma and socioeconomic disadvantage⁵. CAMHS acknowledges that understanding of mental health within Aboriginal communities involves a holistic construct of social, emotional, cultural and spiritual wellbeing⁶.

The Mental Health Act 2014 states the following:

*To the extent that it is practicable and appropriate to do so, the assessment of a person who is of Aboriginal or Torres Strait Islander descent must be conducted **in collaboration with** –*

- a) Aboriginal or Torres Strait Islander mental health workers; and*
- (b) significant members of the person's community, including elders and traditional healers⁷.*

Where it is not possible for the above individuals to be directly involved in a clinical assessment, CAMHS must ensure that the assessment is supported by collateral information / liaison with these individuals. This is particularly important if the Aboriginal child is in the care of a non-Aboriginal person.

All assessment and treatment planning for Aboriginal children in CAMHS must be done in a culturally safe practise and should:

- be appropriate to and consistent with, their cultural and spiritual beliefs⁸;
- use assessment instruments or inventories which are culturally appropriate, and where feasible culturally validated;
- involve significant others from the child's community where consent is given.

Assessment must consider information from:

- the child, family, or
- other key stakeholders (as above), and
- the child's medical record⁹.

All CAMHS staff must undertake the mandatory 'Aboriginal Cultural eLearning' module, as well as any other training packages that may be required to ensure that they are 'culturally competent' to work with Aboriginal children and families.

These resources provide additional information:

[CAHS Aboriginal and Torres Strait Islander Patient Care and Cultural Learning Guidelines Working Together: Aboriginal and Torres Strait Islander Mental Health and Wellbeing Principles and Practice¹⁰](#).

⁴ Working Together: Aboriginal and Torres Strait Islander Mental Health and Wellbeing Principles and Practice, Dudgeon P, Milroy, H & Walker R, Australian Government, Telethon Kids Institute, 2014

⁵ [Clinician's Practice Guide to the MHA](#), p. 21

⁶ Chief Psychiatrist's Standards for Clinical Care, 2015

⁷ MHA – s. 50 and 189

⁸ [Charter of Mental Health Care Principles](#) – Principle 7

⁹ MHA 2014, s. 49, 80

¹⁰ Adams Y, Drew N, Walker R. Principles of Practice in Mental Health Assessment with Aboriginal Australians. In: Dudgeon P, Milroy H, Walker R, editors. 2nd ed. Perth: Telethon Kids Institute (pp 271-288)